

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Philomena D. Arceneaux DOB: 1952-06-11 (age 74) Member ID: MBR-51960

Sex: F

Requesting Provider

Practice: Sorrel Creek Heart Group

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-16

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Echocardiogram report with addendum

SORREL CREEK HEART GROUP - CARDIAC ULTRASOUND

Exam: transthoracic echocardiogram Date of exam: Sept 8 2026

Patient: ARCENEAX, PHILOMENA D DOB Jun 11 1952

LV: mildly dilated. Inferior and inferolateral hypokinesis.

LVEF: 32%

RV normal. Mild MR, mild TR. No effusion.

IMPRESSION: Moderately reduced LV systolic function, LVEF 32%.

Electronically signed: Marisol Teague, MD Sept 8 2026 14:10

ADDENDUM / CORRECTED REPORT Sept 12 2026 09:42

The LVEF in the original report (32%) was carried over in error from the patient's 2024 study during report generation. On review of today's images, LVEF measured by biplane method of discs is 42%. Corrected impression: mildly reduced LV systolic function, LVEF 42%. Wall motion findings unchanged. Referring physician notified by phone.

Electronically signed: Marisol Teague, MD Sept 12 2026

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Electrophysiology consultation

SORREL CREEK HEART GROUP - ELECTROPHYSIOLOGY

Consult date: Sept 10 2026

Mrs. Arceneaux, 74, retired postal worker

Inferior MI Oct 2020, CABG x3 Nov 2020. No chest pain, coronary events or procedures since. Breathless walking up the slope to church and with vacuuming; rests after a block. NYHA III. Never fainted.

TTE Sept 8: LVEF 32%.

Opinion: ischemic cardiomyopathy, LVEF $\leq 35\%$, NYHA III, long on GDMT. Primary prevention ICD is indicated.

Plan: SDM visit, then implant in November.

Dr. Augustin Weatherby

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Shared decision-making visit

SORREL CREEK HEART GROUP - SDM VISIT (ICD)

Sept 14 2026

Patient: Philomena Arceneaux; daughter present

Clinician: Corinne Delahunt, PA-C

An evidence-based ICD decision aid (booklet) was used and reviewed in full.

We talked through benefit, harm, shocks, infection, battery changes, deactivation and the no-device option. She chose the ICD.

C. Delahunt PA-C

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Medication list

Arceneaux, Philomena - meds as of Sept 10 2026

carvedilol 25 mg twice daily (from Dec 1 2020)

sacubitril/valsartan 49/51 twice daily (from 2022; losartan Dec 1 2020 - 2022)

spironolactone 25 mg daily (from Dec 1 2020)

empagliflozin 10 mg daily (from 2023)

aspirin 81 mg; atorvastatin 80 mg

warfarin (mechanical mitral valve? NO - for DVT 2025, plan 12 months)

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Laboratory results

SORREL CREEK LAB Sept 9 2026

Na 138 K 4.4 Cr 1.2 eGFR 46

INR 2.4 (warfarin)

Hgb 12.2

NT-proBNP 940

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Pre-procedure plan

SORREL CREEK EP - PRE-PROCEDURE PLAN

Sept 15 2026

Patient: Arceneaux, Philomena D. MBR-51960

Procedure: ICD, CPT 33249, inpatient admission Nov 16 2026.

Indication: ICM, LVEF 32% (TTE 9/8/26), NYHA III, SDM done 9/14.

Expected stay: 2 midnights - warfarin continued (INR goal 2.0-2.5) with pocket hematoma watch; CKD 3a; age 74 with walker.

Limiting conditions: none (cognition intact, no terminal illness, no AF, stable).

A. Weatherby MD

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